Georg von Westphalen

Uncle FrizzleBob's Krazy Kayfabizzaro Komik Kard Kult Klub: CARDIOLOGY EXPANSION

Fibby Fitz
(Atrial Fibrillation)

STAT: Rhythm: Irregularly Irregular

POWER: Improvise - Ventricles only catch every 3rd beat.
LORE: Decide rate vs. rhythm control, and calculate stroke risk!

Flutterby
(Atrial Flutter)

STAT: Atrial Rate: 300 BPM

POWER: 2:1 Velvet Rope - Ventricular rate locks at 150.
LORE: Vagal maneuvers unmask the flutter waves.

The Wolf
(WPW Syndrome)

STAT: PR < 120ms

POWER: Delta Wave Bypass - Skip AV node delay.
LORE: Do NOT give digoxin or AV-blockers if Fibby Fitz is in the club!

Short cut, suckers!

Vera Fib
(Ventricular Fibrillation)

STAT: Output: ZERO

POWER: Total Blackout - Eliminate pulse.
LORE: Shock first, ask questions later. No pulse, no negotiation.

GAME OVER!

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Mr. Block (1st Degree)

STAT: PR > 200ms

POWER: The Long Wait - Delay, but never drop.
LORE: Benign. No treatment needed.

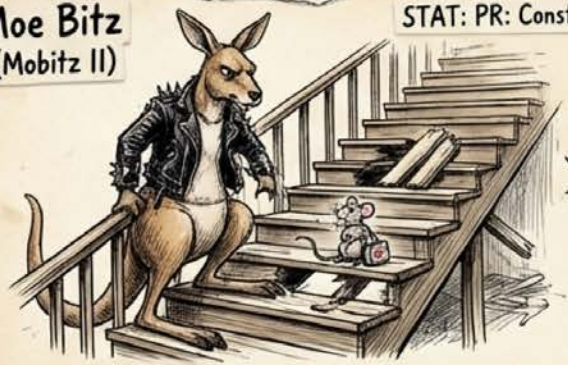
Wenky (Mobitz I)

STAT: PR: Growing...

POWER: Predictable Fatigue - Progressively lengthen PR, then drop QRS.
LORE: Predictable pattern, rarely needs a pacemaker.

Moe Bitz (Mobitz II)

STAT: PR: Constant



POWER: Random Drop - Drop QRS without warning.
LORE: Highly dangerous; often progresses to complete block. Pacemaker required.

The General (3rd Degree)

STAT: P-P and R-R: Independent

POWER: Severed Comms - Atria and Ventricles march to their own beat.
LORE: Bridge therapy required while waiting for the pacemaker!

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STAT!
TACH THE KNIFE
(VENTRICULAR TACHYCARDIA)
STAT: 150-250 BPM
POWER: ORGANIZED SPEED - WIDE QRS COMPLEX TACHYCARDIA.
LORE: Check for pulse! Stable gets meds, unstable gets electricity.
ADENOSINE ASSASSIN
STAT: Half-life: 10 seconds
POWER: HARD RESET - BRIEFLY FLATLINE THE AV NODE.
LORE: Used to unmask hidden rhythms and terminate SVT.
AMI THE AMIODARONE
STAT: Half-life: 40-55 Days
Na
K
Ca
Beta
POWER: CHAOS AGENT - DELAY REPOLARIZATION GLOBALLY.
LORE: Watch for thyroid, lung, liver, and blue-gray skin toxicity!
THE METRONOME TWINS
STAT: Strategy Choice
POWER: AFFIRM TRIAL - CHOOSE RATE OR RHYTHM.
LORE: Rate control is often just as good as rhythm control in AFib.
STAT!
PUNK MEDS!
PUNK MEDS!
RHYTHM RIOT
NO PULSE NO PEACE!
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Stabby Angina

STAT:
ST: Depression

POWER: Demand Ischemia - Attack only during exercise, retreat at rest.
LORE: Predictable and reproducible. Fixed stenosis.

Unstable Stan

STAT:
Trop: +/-

POWER: Plaque Rupture - Attack without provocation.
LORE: Differentiate from NSTEMI by checking Troponin.

The Plumber (STEMI)

STAT:
Door-to-Balloon:
90 Mins

POWER: Transmural Block - Elevate ST segments and kill muscle.
LORE: Complete occlusion. Time is muscle. Get to the cath lab!

Troponin Tim

STAT:
Peak: 12-24 Hrs

POWER: Leakage Indicator - Detect microscopic cardiac injury.
LORE: Sensitive but not specific. Rises in demand ischemia, PE, and renal failure too.

KayfaBINGO!

TROPONIN TERROR!

ISCHEMIA SCREAMIA

CATH LAB OR BUST!

TROPONIN TERROR!

PLAQUE SCREAMIA

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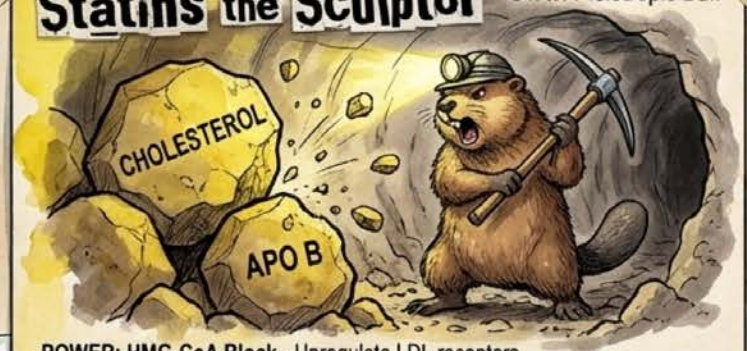
Lady Lipid

STAT: LDL Target: <70 mg/dL

POWER: Vulnerable Plaque - Infiltrate and wait for rupture.
LORE: The most dangerous plaques are vulnerable, not the most occlusive.

Statins the Sculptor

STAT: Pleiotropic Buff



POWER: HMG-CoA Block - Upregulate LDL receptors.
LORE: Watch the CK levels for myopathy/rhabdo! Give high-intensity post-MI.

Aspirin the Ancient

STAT: Duration: 7-10 Days

POWER: Irreversible Acetylation - Disable platelet thromboxane A2 forever.
LORE: Only a fresh platelet transfusion partially reverses her effect.

Ticky the $P2Y_{12}$ Blocker

STAT: DAPT Combo

POWER: ADP Blockade - Prevent platelet aggregation.
LORE: Combines with Aspirin for DAPT after stent placement.

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Nitro the Negotiator

STAT: Preload: Down

POWER: Venodilation - Relieve angina and pulmonary edema.
LORE: Fatal hypotension if combined with PDE5 inhibitors! Avoid in RV infarets.

Beta the Bouncer

STAT: Contractility: Down

POWER: Cardioprotection - Reduce oxygen demand and block remodeling.
LORE: Contraindicated acutely in decompensated HF or severe block.

KayfaBOGGLE!

The Stent Architect

STAT: BMS vs. DES

POWER: Scaffold - Hold open the culprit lesion.
LORE: Drug-eluting stents (DES) prevent restenosis but need longer DAPT.

Bypass the Builder (CABG)

STAT: SYNTAX Score

POWER: New Plumbing - Bypass the obstruction entirely.
LORE: Preferred for left main disease, 3-vessel disease, and complex diabetics.

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Franko the Failing Heart

STAT: EF < 40% (HFrEF)

POWER: Weak Squeeze - Drop output, pool fluid.
LORE: Classify functional severity using the NYHA class.

Eddie Edema

STAT: BNP > 400 pg/mL

POWER: Back-pressure - Flood the alveoli (crackles, orthopnea).
LORE: Differentiate cardiac from pulmonary dyspnea using BNP/NT-proBNP.

ACE the Architect of Afterload

STAT: Mortality: Reduced

POWER: Vasodilation - Drop afterload, stop remodeling.
LORE: Causes a bradykinin-mediated dry cough. Switch to ARB if intolerable!

Arnie the ARNi

STAT: Neprilysin Inhibited

POWER: Dual Strike - Block AT1 receptor AND boost natriuretic peptides.
LORE: Wait 36 hours after stopping ACEi before starting Arnie (Angioedema risk)!

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Aldo the Antagonist

STAT: Potassium-Sparing

POWER: Anti-Fibrotic - Block the mineralocorticoid receptor.
LORE: Reducet mortality in severe HFrEF, but watch for hyperkalemia and gynecomastia!

SGLT2 Stan

STAT: DAPA-HF Proven

POWER: Osmotic Diuresis - Block glucose reabsorption in the kidney.
LORE: The diabetes drug that became a heart failure mortality-reducing hero!

Diggy the Digoxin

STAT: Therapeutic Index: Narrow

POWER: Inotropic Boost - Inhibit Na/K-ATPase to increase intracellular Calcium.
LORE: Hypokalemia dramatically increases the risk of Diggy toxicity!

Lou the Loop

STAT: NKCC2 Blocked

POWER: Flash Flood - Massive diuresis via ascending loop of Henle.
LORE: The front-line decongestive in acute HF. Watch for ototoxicity at high IV doses.

Rx ALERT!
Name
Telemetry

SALT SQUEEZE!

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The Remodeling Monster

STAT: Wall Tension: High

POWER: Dilate & Thin - Change ventricular geometry over time.
LORE: ACEi, Beta-Blockers, and Aldo-antagonists starve this monster.

ICD Ivan

STAT: EF < 35% Indication

POWER: Silent Guardian - Deliver synchronized shock for VT, unsync for VF.
LORE: Primary prevention of sudden cardiac death in optimized HFrEF.

Cardie the CRT

STAT: QRS > 150ms

POWER: Resynchronization - Force Left and Right ventricles to squeeze together.
LORE: Indicated specifically for HFrEF patients with LBBB morphology.

Transplant Passport

STAT: Peak VO_2 < 12

POWER: New Engine - Swap Franko out completely.
LORE: Contraindicated by active infection, malignancy, or severe fixed pulmonary HTN.

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PUNK MEDS

Hyper Hugo

STAT: Stage 2: 140/90+

POWER: Silent Killer – Sustain high pressure indefinitely.
LORE: Urgency = no organ damage (oral meds).
Emergency = organ damage (IV meds).

The RAAS Serpent

STAT: Angiotensin II: MAX

POWER: Constrict & Retain – Spike BP and hoard volume.
LORE: Feeds on low renal perfusion and sympathetic activation. Block it at multiple levels!

Cal the Calcium Blocker

STAT: Dihydropyridine vs Non

POWER: Smooth Muscle Relax – Block L-type calcium channels.
LORE: Dihydropyridines (amlodipine) cause edema. Verapamil slows the AV node and causes constipation.

Thia the Thiazide

STAT: ALLHAT Champion

POWER: Volume Drop – Block NaCl cotransport.
LORE: First-line agent! Causes hypokalemia, hyponatremia, and hyperuricemia (watch for gout).

STAT!

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Alpha Annie

STAT: Resistance: Down

POWER: Arterial Drop - Sudden peripheral vasodilation.
LORE: Great for BPH, but causes brutal first-dose orthostatic hypotension!

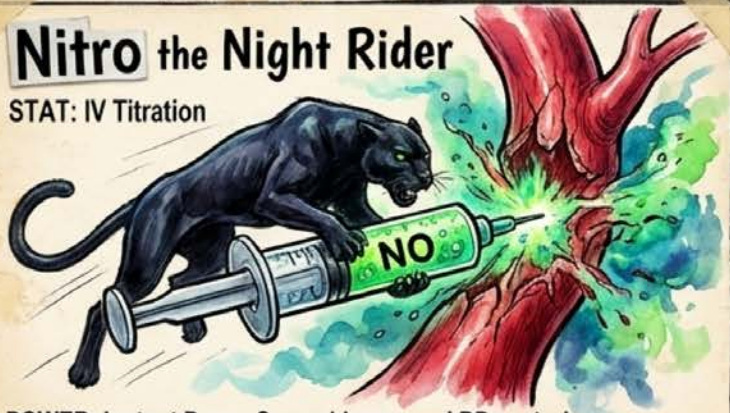
Eclampsia Emergency

STAT: >20 Weeks Gestation

POWER: Systemic Storm - HTN + Proteinuria + End-organ failure.
LORE: Mag sulfate is for seizures, NOT BP control. Give Labetalol or Nifedipine. Cure = Delivery.

Nitro the Night Rider

STAT: IV Titration



POWER: Instant Drop - Second-by-second BP control.
LORE: Perfect for emergencies, but prolonged use risks Cyanide Toxicity! Monitor a-line.

Pheo the Hidden Villain

STAT: Metanephrines: High

POWER: Catecholamine Dump - Episodic HTN, headache, diaphoresis.
LORE: MUST give Alpha-blocker BEFORE Beta-blocker, or risk a lethal hypertensive crisis!

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The Aorta Splitter

STAT: Widened Mediastinum

POWER: Tearing Pain
- Propagate the flap down the aorta.

LORE: Type A (Ascending) = Surgical Emergency.
Type B (Descending) = Medical management (HR/BP control).

The Secondary Suspects

STAT: Refractory to 3 Meds

POWER: Hidden Cause
- Sleep apnea, Renal artery stenosis, Aldosteronism.

LORE: Suspect if BP is refractory to 3 agents or starts before age 30!

Arnie Aortic Stenosis

STAT: Valve Area < 1.0 cm^2

POWER: Fixed Outflow - Restrict LV emptying.

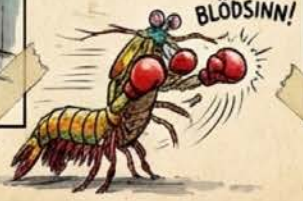
LORE: Symptoms = Angina, Syncope, Heart Failure. Once symptomatic, survival drops fast! (TAVR vs SAVR)

Mitty Mitral Regurgitation

STAT: Holosystolic Murmur

POWER: Volume Overload - Flood the left atrium and lungs.

LORE: Acute MR (papillary rupture) is a shock emergency. Chronic MR is compensated for years.



CARD 1

Mitty Mitral Stenosis

STAT: Diastolic Rumble

POWER: Atrial Stretch - Dilation, AFib, and Pulmonary HTN.
LORE: Almost always rheumatic in origin. Listen for the opening snap!

CARD 2

The Pericardial Knight

STAT: Saddle ST Elevation

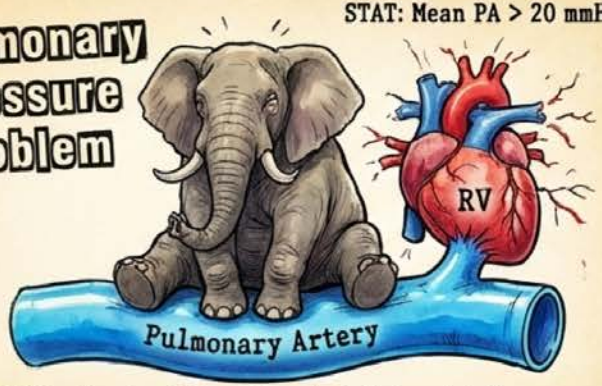
POWER: Friction Rub - Positional sharp chest pain.
LORE: If fluid accumulates fast, Tamponade hits (Beck's Triad, Pulsus Paradoxus). Time for pericardiocentesis!

FIGHT THE SYSTEM!

CARD 3

Pulmonary Pressure Problem

STAT: Mean PA > 20 mmHg



POWER: RV Strain - Cor Pulmonale and right heart failure.
LORE: Group 1 gets advanced pulmonary vasodllators. Groups 2-5 need their root cause fixed!

CARD 4

Cholesterol Carl

STAT: ASCVD Risk Score

POWER: Delivery Boy - Drop lipid packages into the vascular intima.
LORE: Statins first. If LDL targets aren't met in high-risk patients, deploy PCSK9 inhibitors!

NO! NO!
HEART GO BOOM!
FIGHT THE SYSTEM!
FIGHT THE SYSTEM!
BAD BLOOD!
BAD BLOOD!
Stay fluffy!

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Hep the Heparin

STAT: Monitor aPTT

POWER: Antithrombin III Buff - Inhibit Thrombin and Factor Xa.

LORE: Reversible with protamine. LMWH (enoxaparin) requires less monitoring but renally cleared!

The NOAC Squad

STAT: CHA_2DS_2-VASc Guide

POWER: Direct Blockade - Predictable kinetics, no INR checks.

LORE: Replaced warfarin for most AFib/VTE. Reverse dabigatran with idarucizumab, Xa with andexanet alfa.

The Cardiology Attending

STAT: The Synthesis Quest

POWER: Pimping - Demand a fully integrated clinical regimen.

LORE: 68yo diabetic, AFib, EF 32%. Build the optimal regimen, spot the interactions, set the targets!

FrizzleBob's Golden Stethoscope

STAT: Meta-Rule

POWER: Story trumps Lists - Discard rote memorization.

LORE: The science is real, the absurdity makes it permanent. You remember stories, not spreadsheets.

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